# Newton MUN Committees

- WHO (World Health Organization)

# MUN Procedures

- Committee Session
    1) Roll Call
        - All  countries called, delegate responds with “Present” or “Present and Voting”
    2) General Speaker’s List
        - Chosen delegates give opening speech
    3) Moderated Caucus
        - Speaking rules apply
    4) Unmoderated Caucus
        - Informal session, delegates can walk around and talk to other delegates
    5) Drafted Resolutions
        - Done with multiple delegates
    6) Voting
        - Delegates vote for best resolution

# MUN Terminology

- Moderated Caucus
    - Short statements/comments from delegates on sub-issues
    - If someone wants to speak on the subject, they must raise their placard and wait for the chair to call on them
- Unmoderated Caucus
    - Free movement
    - Talk to other delegates
    - Form alliances
    - Brainstorm resolutions

- Floor
    - Opportunity to speak
- Motion
    - Suggest an action
        - Extend debate time
        - Vote on resolutions
- Second
    - Agree with another delegate's motion
- Objection
    - Disagree with another delegate's motion
- Abstain
    - If a delegate is unsure about their vote they can choose not to say yes or no definitively
        - Does not fully agree
        - Agrees enough not to vote against

# MUN Points

- Question or suggestion made by a delegate to the chair
- Usually regarding their comfort
- Types of points :
    - Point of Order
        - Objection to a procedure
        - The chair has made a mistake
        - Parliamentary Procedures are not done correctly
        - Must be made
        - Must say point of order whilst raising their placard
        - May interrupt a speaker who has the floor

- Point of Inquiry
    - For clarification
- Point of Information
    - To add onto another speaker statement/resolution
    - To bring down the resolution
    - Must be formulated as a short question
    - May NOT interrupt speaker who has the floor
    - Must be directed to the chair, even though it is aimed at the speaker
    - Max 2 times
- Point of Personal Privilege
    - When a delegate feels uncomfortable
    - Room too hot / too cold
    - May interrupt speaker to request for change in atmosphere
- Point of Parliament Inquiry
    - Point of info directed at chair regarding the rules of procedure

# WHO Overview

- Committee Summary : The World Health Organization (WHO) serves as the directing and coordinating authority for international health within the United Nations system. Tasked with the ambitious goal of ensuring "Health for All," the committee operates at the intersection of science, policy, and humanitarian advocacy. When addressing The Question of Global Health Standards in Vulnerable and Undeveloped Regions, the WHO's primary objective is to bridge the widening gap between medical innovation and grassroots accessibility. This involves navigating the complexities of Universal Health Coverage (UHC), strengthening fragile healthcare infrastructures, and establishing ethical guidelines for medical intervention in regions plagued by conflict, poverty, or environmental crises.
- Agenda :
    - Topic 1 - The Question of Global Health Standards in Vulnerable and Undeveloped Region
- Committee Allocations
    - Afghanistan
    - Australia
    - Austria
    - Brazil
    - Chad
    - France
    - Germany
    - Japan
    - Somalia
    - South Sudan
    - Syria
    - United Kingdom
    - Uganda

# Key Terms (Topic 1)

| *Term* | *Category* | *Definition* |
|---|---|---|
| **Social Determinants of Health (SDOH)** | ***Economic & Structural Concepts*** | ● Non-medical factors that influence health outcomes<br>● **Examples**; income, education, access to clean water, and housing conditions |
| **Health Inequity** | ***Economic & Structural Concepts*** | ● Differences in healthcare that are unjust, avoidable, and unnecessary |
| **Health Inequality** | ***Economic & Structural Concepts*** | ● Mathematical differences in health status |
| **Universal Healthcare Coverage** | ***Economic & Structural Concepts*** | ● The goal for all people to receive the health services they need without suffering financial hardship |
| **International Health Regulations (IHR)** | ***Standards & Frameworks*** | ● Legally binding instrument of international law → requires reporting certain disease outbreaks and public health events to the WHO<br>● |
| **The 10/90 Gap** | ***Standards & Frameworks*** | ● The statistical phenomenon where only 10% of global health research funding is spent on diseases that cause 90% of the world's health problems |
| **Minimum Essential Levels** | ***Standards & Frameworks*** | ● The "floor" of healthcare services that must be provided even in resource-poor settings, often sparking debate over whether standards should be universal or context specific |
| **DALY (Disability-Adjusted Life Years)** | ***Disease & Vulnerability*** | ● Measure of overall disease burden<br>● Expressed as a number of years lost due to ill-health, disability, or early death<br>● **Formula** : DALY = YLL + YLD<br>(YLL = Years of life lost) (YLD = Years lived with disability) |
| **Endemic** | ***Disease & Vulnerability*** | ● Common struggle of vulnerable regions<br>● Constantly present<br>● **Example** : Malaria |
| **Epidemic** | ***Disease & Vulnerability*** | ● Sudden disease outbreaks<br>● Garners lots of international attention |
| **Neglected Tropical Disease (NTDs)** | ***Disease & Vulnerability*** | ● A group of communicable diseases that thrive in tropical/subtropical conditions<br>● Disproportionately affects the "bottom billion" people living in poverty |
| **Brain Drain** | ***Ethical & Practical Barriers*** | ● The emigration of highly trained professionals from developing nations to wealthier countries<br>● Leaves vulnerable populations understaffed |
| **Medical Sovereignty** | ***Ethical & Practical Barriers*** | ● The right of a nation to govern its own health policies and clinical trials without undue influence from foreign NGOs or pharmaceutical giants |

| | | |
|---|---|---|
| **Cold Chain Logistics** | ***Ethical & Practical Barriers*** | • The temperature-controlled supply required to transport vaccines and medicines to remote or undeveloped areas<br>• Without this global standards remain theoretical |
| **TRIPS Agreement** | ***Economic & Pharmaceutical Barriers*** | • AKA. Trade-related aspects of intellectual property rights<br>• Major point of contention regarding the production of generic drugs vs. name-brand patents in developing nations |
| **Essential Medicines List (EML)** | ***Economic & Pharmaceutical Barriers*** | • A published list (by the WHO) of the most efficacious, safe and cost-effective medicines that healthcare systems should have available |
| **Out-of-Pocket Expenditure (OOPS)** | ***Economic & Pharmaceutical Barriers*** | • Direct payments made by individuals to healthcare providers<br>• Pushes families in undeveloped regions into extreme poverty |
| **Tiered Pricing** | ***Economic & Pharmaceutical Barriers*** | • A strategy where pharmaceutical companies sell drugs at different prices in different countries focused on their economic status |
| **Task Shifting** | ***Infrastructure & Delivery*** | • Moving specific tasks from highly trained healthcare professionals (e.g. doctor) to those with less training (e.g. community healthcare worker) to maximize human resources |
| **Last Mile Delivery** | ***Infrastructure & Delivery*** | • The final leg of the supply chain, often the most difficult in underdeveloped regions due to a lack of roads, electricity, and security |
| **Point-of-Care (POC) Diagnostics** | ***Infrastructure & Delivery*** | • Medical testing at or near the site of patient care (like rapid malaria tests)<br>• Essential where lab infrastructure is limited or non existent |
| **eHealth/mHealth** | ***Infrastructure & Delivery*** | • The use of mobile devices and digital technology to deliver health services and information in remote areas |
| **Medical Pluralism** | ***Governance & Ethics*** | • The employment of more than one medical system within a population<br>• Examples; combining western “biomedicine” with traditional or indigenous healing |
| **Hegemony in Health** | ***Governance & Ethics*** | • The dominance of western medical standards over local indigenous knowledge<br>• Often leading to “one size fits all” approach that fails |
| **Vertical Programs** | ***Governance & Ethics*** | • Funding focused on one specific disease<br>• Examples; Polio research funding |
| **Horizontal Programs** | ***Governance & Ethics*** | • Funding focused on strengthening the overall healthcare system |
| **Double Burden of Disease** | ***Governance & Ethics*** | • When a region faces the “old” problems of infectious diseases while simultaneously seeing a rise in “western” lifestyle diseases like diabetes and |

| | | |
|---|---|---|
| | | hypertension |
| **Zoonotic Spillover** | ***Clinical & Scientific Metrics*** | ● The transmission of a pathogen from animals to humans<br>● High risk in regions with close human-wildlife proximity and poor sanitation |
| **Antimicrobial Resistance (AMR)** | ***Clinical & Scientific Metrics*** | ● A crisis exacerbated in undeveloped regions by the availability of “over-the-counter” antibiotics without prescriptions and substandard medications |
| **Case Fatality Rate (CFR)** | ***Clinical & Scientific Metrics*** | ● The proportion of people from a specific disease who die from it<br>● In vulnerable regions, the CFR is often significantly higher for the same disease due to a lack of supportive care |
| **Stunting** | ***Clinical & Scientific Metrics*** | ● Low height for age<br>● Chronic malnutrition |
| **Wasting** | ***Clinical & Scientific Metrics*** | ● Low weight for height<br>● Acute malnutrition |
| **Fragile States** | ***Clinical & Scientific Metrics*** | ● Countries where the government cannot or will not deliver core services (like healthcare) to the majority of its people |
| **Medical Neutrality** | ***Clinical & Scientific Metrics*** | ● The ethical principle that healthcare workers should be allowed to treat the sick regardless of political affiliation<br>● Especially relevant in conflict zones |
| **Health Diplomacy** | ***Clinical & Scientific Metrics*** | ● When nations use health interventions as a tool to improve relations or exert “soft power” influence |
| **Internal Displacement (IDPs)** | ***Clinical & Scientific Metrics*** | ● Populations forced to flee their homes but who remain within their countries borders<br>● Often falling in a “healthcare gap” where they lack the rights of refugees but have no access to local services |

# Afghanistan

- Healthcare systems heavily reliant on donor funding – which has made a massive impact in expanding care
- In 2003, 9% of the Afghan population were able to access basic health services
- In 2002, 40% of healthcare facilities had no female health workers – straining access to care for females
- In 2002, ⅓ of districts had 30,000+ inhabitants per facility
- In 2010:
    - Infant mortality : 77/1000
    - Under 5 mortality : 97/1000
    - Maternal mortality ratio : 372/100,000
    - Antenatal care coverage : 68%
    - Delivery by skilled birth attendants : 34%
    - Full immunization coverage : 37% (2008)
    - Access to primary health services within 1-2 hour driving distance : 90% (GOAL)
- Only 54% monitored blood pressure during pregnancy
- Low utilization of healthcare facilities due to a lack of trust and resources
- Proposed the establishment of a new IQHC unit
    - To promote and drive activities
    - Activities designed to achieve high quality services, coordinate existing quality programs, monitor their efficacy and impact, and oversee the strategic commissioning of further work
- Needs for new pilot programs (designed specifically for Afghanistan's unique circumstances)
- National policy used WHO's guide : 'Quality of Care : A process for making strategic choices in health systems' as a major support point for the creation of legislation
- In 2011: National Strategy:
    - Standardized the definition of quality in the Afghan context
        - 'A quality healthcare system is client-centered, equitable, available, appropriate, safe, consistent, effective, timely, and efficient'
    - Defined strategic objectives
        - Improving patient safety
            - DRAFTED Patient Charter of Rights - MoPH
        - Providing client-centered services
        - Strengthening the data recording and reporting system
        - Improving clinical practices
        - Building capacity of the system to continuously improve
        - Improving health outcomes in priority areas of maternal and neonatal care, child and adolescent health, nutrition, disability, mental health, and communicable diseases
            - Focus on high impact - low resource objectives

    - Created a strategic implementation framework
    - Created a detailed 5-year operational plan
    - Developed a measurement and data collection strategy
    - Supported the integration of the strategy into revised MoPH documents
    - Enforced routine performance assessments
- Majority of the population is Islam
    - Leads to difficulties accessing care for females in non-emergencies
        - Although male doctors may treat a female in an emergency, they must never be alone in a room and requires another trusted woman

# Taliban

- Personal hygiene is nearly impossible with salons being banned by the Taliban
- The Taliban has been described as enforcing a ‘gender apartheid’ by the UN
- 90% of all women's “protests” took place inside due to the restrictions on freedom and peaceful protests.
- Taliban continues to engage in arbitrary arrests, forced disappearances, torture, and other ill treatment and unjust killings of former government employees, human rights defenders, journalists, and critical voices
    - Hundreds of arrested journalists sentenced to death in prison
- Female inequalities :
    - Reduced wage to $70 per month
        - Cannot work for the government or numerous other job sectors
    - May not attend secondary school (grade 6+)
        - Banned from attending medical school
            - Contradicts Islam mandates of women being treated by women unless it is an emergency
    - Must not travel more than 72km or be seen in public without mahram *(A mahram is a male family member with whom marriage is permanently prohibited in Islam due to blood relation, marriage, or fosterage)*
    - Females remain barred from participating in the UN or any NGOs
- Women are being detained for non-compliance with hijab mandates
    - Reports of degrading treatments, torture, and rape
- UN urges to recognise gender apartheid as a crime under international law
    - Germany, Australia, Canada, & Netherlands announced legal action before the ICJ against Afghanistan for violations of CEDAW by the Taliban
- Taliban banned entry of the special rapporteur on Afghanistan
- UN Special Procedures mandate holders highlighted that an avenue for justice was “virtually non-existent” in Afghanistan

# Healthcare Indicators

- **Health status**
  - Mortality
  - Morbidity
  - Fertility
- **Risk factors**
  - Nutrition
  - Environmental
  - Behavioural
- **Service coverage**
  - Reproductive & new born
  - Immunization
  - Treatment coverage
- **Health systems**
  - Health workforce
  - Infrastructure
  - Financing

# Health Status

### Mortality :

- Life expectancy
  - Afghanistan : 59-66 years
  - Global Goal : 80 years
- Maternal mortality ratio
  - Afghanistan : 570/100,000
  - Global Goal : <70/100,000
- Infant mortality rates
  - Afghanistan : 70/1,000
  - Global Goal : <12/1,000
- Under-5 mortality rates
  - Afghanistan : 60/1,000
  - Global Goal : <25/1,000
- Cause-specific mortality rates
  - Afghanistan : NCDs = 49.7% || Communicable = 32.9%
  - Global Goal : Reduce by ⅓ / 33.3%

### Morbidity :

- Incidence & prevalence of HIV
  - Afghanistan : 0.1% (13,000) → -90% = 1,300 (0.01%)
  - Global Goal : Reduce by 90%

- Incidence & prevalence of Tuberculosis
    - Afghanistan : 180/100,000 = 0.18% → -80% = 36/100,000
    - Global Goal : Reduce by 80%
- Incidence & prevalence of Malaria
    - Afghanistan : 17.76/1,000 = 1.776% → -90% = 1.776/1,000 (0.1776%)
    - Global Goal : Reduce by 90%
- Incidence & prevalence of Diabetes
    - Afghanistan : 12% (1.9 million)
    - Global Goal : Prevent increases

### Fertility :

- Total fertility rate
    - Afghanistan : 4.84 children per woman
    - Global Goal : 2.1 children per woman
- Adolescent birth rate
    - Afghanistan : 64/1,000 women
    - Global Goal : 2.5/1,000 women

# Risk Factors

### Nutrition :

- Stunting in children
    - Afghanistan : 43% → -40% = 25.8%
    - Global Goal : Reduce by 40% (by 2030)
- Wasting in children
    - Afghanistan : 9%
    - Global Goal : <3% by 2030
- Anemia in women
    - Afghanistan : 46% → -50% = 23%
    - Global Goal : Reduce by 50%
- Adult obesity rates
    - Afghanistan : 18.2%
    - Global Goal : Reduce by 6%

### Environmental :

- Access to safe drinking water
    - Afghanistan : >30% lacking
    - Global Goal : 100% access
- Use of clean household fuels
    - Afghanistan : 90% use of non-clean fuels
    - Global Goal : 100% access
- Exposure to air pollution (PM2.5)
    - Afghanistan : 10x global guideline
    - Global Goal : 5□□/□3 guideline

### Behavioural :

- Tobacco use

    - Afghanistan : >30%
    - Global Goal : <5% (by 2040)
- Alcohol consumption per capita
    - Afghanistan : 0.01 litres → 0.002 litres
    - Global Goal : Reduce by 20% by 2030
- Physical inactivity
    - Afghanistan : 40.3% of adults → 30.2249%
    - Global Goal : Reduce by 15%

# Service Coverage

### Reproductive and Newborn :

- Antenatal care coverage
    - Afghanistan : 21.5% recieved 4 appointments
    - Global Goal : at least 8 appointments (1st within 12 weeks of birth)
- Births attended by skilled health personnel
    - Afghanistan : 61%
    - Global Goal : 100% attendance (currently 87%)

### Immunization :

- Coverage of DTP3 Vaccine
    - Afghanistan : 64%
    - Global Goal : 90%
- Coverage of Measles (MCV2) Vaccine
    - Afghanistan : 42%
    - Global Goal : 95%
- Coverage of HPV Vaccine
    - Afghanistan : 0%
    - Global Goal : 90%

### Treatment Coverage :

- Antiretroviral Therapy (ART) for HIV
    - Afghanistan : 11%
    - Global Goal : 100%
- TB treatment success rates
    - Afghanistan : 94.5%
    - Global Goal : 100%
- Hypertension control
    - Afghanistan : 30% → 7.5%
    - Global Goal : Reduce by 25%

# Health Systems

### Health Workforce :

- Density of physicians (per 10,000)
    - Afghanistan : 2.75
    - Global Goal : 24
- Density of physicians, nurses, and midwives (per 10,000)
    - Afghanistan : 9.8
    - Global Goal : 44.5

## Infrastructure :

- Hospital bed density
    - Afghanistan : 4/10,000
    - Global Goal : 44/10,000
- Availability of essential medicines
    - Afghanistan : 70%
    - Global Goal : 100%

## Financing :

- Total health expenditure (% of GDP)
    - Afghanistan : 0.18%
    - Global Goal : 6.5%
- Proportion of the population experiencing “catastrophic” out of pocket health spening
    - Afghanistan : 45%
    - Global Goal : <1%

# Opening Speech

Honorable Chair, Distinguished Delegates, Afghanistan is trapped between a rock and a hard place, with the need to meet basic standards of neutral, accessible, and quality care without triggering further political tensions. Unlike many western nations, Afghanistan remains incredibly vulnerable to the threat of zoonotic and infectious diseases that continue to spread due to poor sanitation and reliance on livestock in rural regions. The delegation of Afghanistan believes this committee must develop a stepped model of health goals that consider external factors such as geographical disparities and political conflict to provide a realistic and actionable plan that is applicable in a variety of contexts. Furthermore, Afghanistan believes in the transition towards community-based care and supports the prospect of bi-monthly village health kits provided to rural regions to prevent the need for specialized care. Finally, Afghanistan strongly urges nations such as Australia and Germany to build partnerships with vulnerable countries like Afghanistan to provide resources, funding, and staffing in exchange for unique national goods and exports. This delegation is eager to further develop an understanding of universal stances on global healthcare and is open to a wide range of ideas. Thank you.

(65s)

# Position Paper

Afghanistan - WHO

# Resolution Ideas

- Incentivize highly trained professionals from developed healthcare systems to migrate to less developed nations to support their healthcare systems
    - Free visas
    - Wage and housing supported by a Global Health Fund
- Village health kits (supplied every 2 months)
    - Supplies include;
        - Any required essential medicines
        - Blankets and non-perishable food items
            - Baby food & formula
            - Feminine hygiene products —> period underwear for reusability
            - Clean modesty covering
        - Bandages & medical tape
        - At least one telemedicine device per 1,000 people
            - Connected via non-terrestrial internet (Satellite)
        - Portable washing machine or device
- Local training programs to prevent the need for hospitals in non dire situations
    - Training on basic stitches (provide fake skin for practice)
    - Training on life saving techniques (CPR)
    - Training on signs of dying
    - Training on chronic illness & disability care
    - Training on pregnancy risks & birthing
    - Training on prenatal, neonatal, and antenatal care
    - Training on basic human development
- Standards customized to the circumstances
- Defining vulnerable
    - Level 1 : at risk of having healthcare interrupted by an external factor (such as (inter)national conflict, weather, or a suspension in trade)
    - Level 2 : has an inconsistent healthcare system with a lack of high-quality care
    - Level 3 : has a dangerous healthcare system with a lack of highly trained professionals and a lack of resources
    - Level 4 : under active attack from a national or international force → immediate referral to the UNSC under the "Responsibility to Protect" protocols
- Resolve the ban by the Taliban on women's (secondary and higher) education – specifically healthcare training
- Provide minimum standards for essential medications available per person – resolve inequities by establishing global partnerships between high efficacy healthcare systems and at risk systems.
- Offer a deal to the Taliban, if participating in any forum of the UN or an NGO, they must uphold all national and international laws, in the case that this is not immediately upheld – weekly migration groups supported by UN volunteers will assist in safely relocating high risk civilians (including high risk patients) in zones particularly vulnerable to the Taliban to partner countries (determined during the committee)
- A UN facilitated transition authority (made up of several different national volunteers) for the purpose of resolving conflicts in Afghanistan
    - Temporary international residence of multiple nations to mediate conflict and take over from the Taliban and appoint new and trustworthy national leaders with

democracy determining the style of government and necessary positions of power
- Free visa program for all Afghan citizens in dire health (level 3+) → partnership with a developed nation (ideally within close proximity)
- Taliban members on duty are not welcome in public healthcare facilities to maintain the safety and peace of patients → no weapons are permitted
    - Entrances are secured by UN volunteers
- Patient risk levels :
    - Level 1 : can be treated virtually or via a local healthcare worker
    - Level 2 : extended illness impacting everyday life and preventing the enacting of daily tasks
    - Level 3 : severe illness requiring hospitalization —> can request for a referral to another healthcare facility capable of more effective treatment
    - Level 4 : critically ill, possibility of death —> possible health based migration
    - Level 5 : actively dying and capable of recovery with high quality care —> relocation to a capable healthcare facility (in another country if necessary)
    - Level 6 : disabled with no ability to survive under current circumstances —> health mandated permanent migration for a quality life
- All medical facilities must have natural disaster plans regarding possible hazards like floods and earthquakes

# Clause Ideas

# Motions

- **Motion to extend speaking time to 90 seconds**
- **Motion for __ minutes unmoderated caucus**
- **Motion for __ minutes moderated caucus on the topic of __**
- **Motion for a consultation of the whole for __ minutes on the topic of __**
- **Motion to open debate on resolution __**
- **Motion to adjourn session**
- **Motion to close debate on resolution __**

# Topics

- Leveraging AI and Digital Health to Bridge Care Gaps
- Developing Infrastructure Deployment for Resource-Poor Regions
- Establishing WHO-Led Performance Monitoring
- Implementing Incremental Standard Realization
- Establishing Situational Risk Mapping
- Focusing on Community-Based Care & Localized Training
- Mitigating "Brain Drain"
- Neutral Healthcare Zones & Peacekeeping
- Securing Critical Medical Infrastructure
- Enforcing Substance Regulation in Crisis
- Enabling Medical Migration & Specialized Visa Programs
- Mandating International Intervention for Equitable Care
- Implementing Integrated Mental Health Paradigms

# POIs

**This delegate would like to ask through the chairs,**

- How does the delegate plan on ___?
- How does the delegate aim to __?
- Why does the delegate believe __ is necessary?
- What funding does the delegate intend on receiving?
- How does the delegate believe this will have a positive impact?
- How can nations be sure this is mutually beneficial and won't result in a national debt?
- Does this solution address both mental and physical health and how?
- Does this account for regions impacted by political conflict?
- How can the committee ensure this funding is sustainable? Or when will it terminate?
- How does the delegate define vulnerability?
- What constitutes and undeveloped nation?
- What supplies beyond essential medications do all communities need?
- Are these standards providing equitable or equal access to healthcare?
    - In vulnerable communities this difference is crucial.
- How does this address the staffing shortage?
- How can nations engage in transformation from within?

# POCs

**This delegate would like to ask through the chairs,**

- What does the delegate mean by __?
- Can the delegate elaborate on the impact of __?

- **POI - point of information**
    - **This delegate would like to ask through the chairs ___**
    - Follow up (wait for it to be granted, 1x only)
- **POC - point of clarification**
    - **This delegate would like to ask through the chairs ___**

# Topics

# Other Speeches

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## Leveraging AI and Digital Health to Bridge Care Gaps

Honorable Chairs, Esteemed Delegates, the use of AI and digital health platforms have already shown incredible progress in bridging access gaps across geographically complex territories with mobile services increasing Afghan access to basic healthcare by 11%. Today, the committee is faced with a complex decision regarding the protection of privacy and safety in the use of health focused digital platforms. The delegation of Afghanistan strongly believes in the responsible implementation of AI healthcare systems, with limits on advice, and forced referrals in severe circumstances. By implementing restrictions, this committee will bridge access gaps and provide equitable access to basic healthcare and simple consultations services. Furthermore, considerations must be made for the censorship and interference with health services therefore, the WHO must mandate that all telemedicine devices are available via satellite. This delegation is committed to ensuring equitable access for all citizens of Afghanistan and of the world. Thank you, this delegate yields the floor to questions.

(Time : 60s)

## Developing Infrastructure Deployment for Resource-Poor Regions

Honorable Chairs, Esteemed Delegates, one of the biggest hurdles in achieving global health standards is the lack of resources and funding available to many nations, this means even those who can afford care are unable to receive it due to the lack of technology, space, or staff. Afghanistan believes that in order to rectify the disparities in resources, nations must commit to allocating a minimum of 2.5% of the GDP towards national healthcare and infrastructure with a goal of increasing to 5% as proposed by the The Civil Society Engagement Mechanism for UHC2030. Furthermore, the delegation of Afghanistan urges all economically advanced nations to engage in donations towards a global fund for developing and providing village health kits in rural regions of the world. Kits should include at a minimum; blankets, clean clothes, bandages, first aid kits, non perishable food, baby food and formula, and essential medications. Afghanistan believes that with such support and assistance from other countries, vulnerable nations like ours can access the fundamental human right that is quality care. Thank you, this delegate yields the floor to questions.

(Time : 60s)

# Establishing WHO-Led Performance Monitoring

Honorable Chairs, Esteemed Delegates, the tracking of health system progress is necessary to reassess goals and provide sufficient foreign aid to vulnerable nations. That being said, Afghanistan is in a unique position where WHO metrics are often hindered by the interference by the Taliban thus, as a nation we condemn the Taliban's actions and encourage a legally binding clause mandating the visitation of multi-national WHO representatives to each nation of concern every 3 years to gather metrics and review current goals and deadlines of the respective goals. The delegate of Afghanistan also believes that the provision of UN volunteers stationed within and outside healthcare facilities in conflict will successfully protect the civilian population while maintaining a politically neutral zone. If the protection is unsuccessful the media attention driven by the involvement of the UN will mandate UNSC action and may even mediate conflict nations between governing bodies and civilians. The delegation of Afghanistan believes that the reliance on the WHO to guide policy innovation and protection of neutrality is critical to the protection of high risk civilians such as women and children.

# Implementing Incremental Standard Realization

Honorable Chairs, Esteemed Delegates, the expectation for nations to uphold the same health standards across the globe is a "one size fits all" approach and fails to consider the various external influences in each context. The delegate of Afghanistan firmly believes in the establishment of stepped healthcare goals to address the critical needs of struggling nations, by categorizing nations and their healthcare systems, the WHO can begin to collaborate, plan interventions, and provide support to ensure universal health coverage becomes a timely reality. Furthermore, Afghanistan believes several areas must be considered to determine the risk and vulnerability of a nation and their healthcare systems – namely, political consensus and peace, economic stability, per capita income, prevalence of specific outbreaks, plans for improvement, vulnerability to natural hazards, upholding of human rights, access to basic health services, religious and cultural influence, and the alignment with both national and international law. Countries that are deemed critically deficient on 5 or more of these scales should be categorized as a level 4 risk, requiring immediate intervention by the UNSC to aid in combating the unsatisfactory results. A level 1 would be a stable healthcare system ranking average or above average across all scales which may trigger requests for foreign medical aid and/or policy guidance. The delegation of Afghanistan yields the floor to questions.

(Time 75s)

# Establishing Situational Risk Mapping

Honorable Chairs, Esteemed Delegates,

## Focusing on Community-Based Care & Localized Training

Honorable Chairs, Esteemed Delegates,

## Mitigating "Brain Drain"

Honorable Chairs, Esteemed Delegates,

## Mandating Neutral Healthcare Zones & Peacekeeping

Honorable Chairs, Esteemed Delegates,

## Securing Critical Medical Infrastructure

Honorable Chairs, Esteemed Delegates,

## Enforcing Substance Regulation in Crisis

Honorable Chairs, Esteemed Delegates,

## Enabling Medical Migration & Specialized Visa Programs

Honorable Chairs, Esteemed Delegates,

## Mandating International Intervention for Equitable Care

Honorable Chairs, Esteemed Delegates,

## Implementing Integrated Mental Health Paradigms

Honorable Chairs, Esteemed Delegates,

# Notes

- Goals for health indicators
    - Level 1
    - Level 2
    - Level 3
    - Level 4
- How can bordering nations protect themselves from infectious diseases?
- How can we consider developed nations with struggling healthcare systems?
- How can we implement safe eHealth platforms with a focus on AI utilization for non-critical injuries and referral advice for emergencies?
- How can the WHO oversee progress in politically tense nations unwilling to provide national metrics?
- What health services should be prioritized?
- To what extent should severe illness and injury be treated palliatively in resource poor contexts to reduce resource depletion when the prognosis is bleak?
- What statistics indicate a change in vulnerability?
- What is sufficient staffing?
- What is sufficient funding?
- Should the WHO manage the global fund or simply oversee any issues while members control the fund?
- What standards should apply to all nations?
    - What standards must be broken into steps?
- What happens when a country withdraws their funding and/or resources?
    - How can we make a sustainable and mutually beneficial trade deal?

Vulnerable = “a nation that is at risk of not being able to provide basic healthcare services to all citizens.”

Undeveloped = “a nation lacking in a combination of; legislation, infrastructure, resources, funding, and/or staffing”